



From resistance to progressivity: The sequential organisation of refusal management in Taiwanese long-term home-based dementia care

Chin-Hui Chen*, Chia-Yu Li

National Pingtung University of Science and Technology, Taiwan Department of Modern Languages, No. 1, Shuefu Road, Neipu Township, Pingtung County, 91201, Taiwan

ARTICLE INFO

Article history:

Available online 8 April 2026

Keywords:

Conversation analysis
Dementia care communication
Refusal management
Home-based care
Taiwan

ABSTRACT

Dementia care communication frequently encounters refusals, particularly in home-based settings where institutional authority must be negotiated. Departing from clinical models that treat resistance as a symptom, this study approaches refusals as socially situated interactional events. Drawing on a corpus of naturally occurring interactions between a professional caregiver and an older woman in Taiwan, the analysis employs Conversation Analysis to examine how refusal management is accomplished as a sequential practice. The caregiver maintains task progressivity through deferrals, downgraded directive renewals, and trajectory modifications, while silence operates as interactional non-uptake occasioning reformulations or activity shifts. Confucian-based expectations of filial respect and face maintenance are further indexed to pursue cooperation while preserving the recipient's dignity, extending person-centred care theory in this Taiwan-based context. © 2026 Elsevier Ltd. All rights are reserved, including those for text and data mining, AI training, and similar technologies.

1. Introduction

1.1. Dementia and the centrality of communication

Dementia is a health condition that is marked by progressive impairments in memory, reasoning, or communication abilities so those with dementia may fail to accomplish everyday activities as a result (World Health Organization, 2025). As the condition advances, it increasingly interferes with how those with dementia can participate in familiar communicative routines (Sabat, 1994). These disruptions in interactions as experienced by those with dementia have drawn academic attention to issues related to dementia and communication.

The global rise in dementia diagnoses has closely followed population ageing trends. Between 1990 and 2016, the number of people living with dementia more than doubled, reaching 43.8 million (Nichols et al., 2019). By 2021, that figure had grown to approximately 57 million, with nearly 10 million new cases each year (World Health Organization, 2025). These figures underscore not only the growing healthcare and economic burdens (Frisoni et al., 2023; Jones et al., 2024), but also the pressing need to better understand the communicative and interactional dimensions of dementia care.

* Corresponding author.

E-mail addresses: irenechen@mail.npust.edu.tw (C.-H. Chen), Zen20000531@gmail.com (C.-Y. Li).

In Taiwan, demographic ageing has accelerated in recent years. As of 2023, 18.35% of the population was aged 65 and over (Statista, 2024), and among this group an estimated 7.64% were living with dementia (TADA, 2025). With prevalence expected to rise further, pressures on both formal care systems and dementia care professionals are mounting. In response, Taiwan has expanded its long-term care system, particularly in the area of home-based services. These care arrangements involve more than the mere provision of physical assistance; they are fundamentally enacted through the everyday interactions between care workers and recipients. Within these settings, communication is constitutive of the care process itself and it is the primary medium through which care is carried out, consent is negotiated, and relationships are sustained. This study examines these communicative dynamics by investigating a case within Taiwan's long-term home care system, with a particular focus on how routine interactional processes both shape, and are shaped by, the practicalities of dementia care.

1.2. Home-based dementia care in Taiwan: policy, culture, and interactional challenges

Because of growing concerns over population ageing and the rising number of individuals living with dementia, the Taiwanese government introduced the Long-Term Care 2.0 Plan to develop a more accessible, affordable, and community-oriented care system (Yang et al., 2020). A key component of this social care system has been the expansion of home-based services for older adults with functional impairments, including those diagnosed with dementia from age 50 onward. The steady growth in service uptake reflects not only policy reach but also longstanding cultural values in Taiwan that prioritise ageing in familiar environments. This preference is particularly salient in dementia care, because the continuity of everyday routines and spaces can help stabilise emotional and cognitive well-being of those with dementia (Yang et al., 2020).

Rather than turning to institutional care, many families in Taiwan now rely on long-term home-care arrangements that allow individuals with dementia to remain at their own home that is imbued with personal meaning. However, these domestic environments might shift the nature of care from mainly task delivery to perhaps also a form of ongoing negotiation (Fæø et al., 2019). For instance, communication considerations related to issues of autonomy, cooperation, and resistance in care communication processes can emerge, because care users remain in control of their home space and caregivers must be attuned not only to physical needs but also to the relational dimensions that shape the communication (Chen, 2024). In this context, caregiving involves far more than supporting daily routines or ensuring physical safety (Thomé et al., 2003; Li and McLaughlin, 2012; Henriksen et al., 2009). It also demands emotional labour, attentiveness to affective cues, and careful management of interactional rapport (Bailey et al., 2015; Kim et al., 2024).

These communicative challenges that caregivers might experience in delivering long-term home care services can become even more visible when the care communication involves disruptions from care users' cognitive decline. In dementia care communication, caregivers must learn to respond flexibly if dementia care users' speech styles are fragmented or ambiguous in nature. Furthermore, in such care settings, caregivers also need to sustain emotional connection, with dementia care users, for instance, by adjusting their strategies to encourage participation and enhancing understanding (Levinson et al., 2010; Gjellestad et al., 2023).

In addition to the communication challenges discussed above, another recurrent interactional issue in dementia care is the negotiation of care refusals. People with dementia may resist assistance with activities such as eating, bathing, or taking medication, and prior work suggests that such resistance is not always straightforward defiance but may be associated with confusion, discomfort, or attempts to retain a sense of control (O'Brien et al., 2020; Pergolizzi et al., 2019). Communication in the face of resistance can carry emotional weight and ethical complexity, and caregivers may need to balance task completion with sensitivity to care recipients' preferences and dignity (Caputo, 2021; Mortensen et al., 2021).

Accordingly, this study treats refusals as socially communicative actions, defined here as displayed non-acceptance produced at points where a response to a caregiving directive action (including offers, proposals, requests, and instructions) is relevant. The criteria used to identify refusals in the data (including recurrent formats and what follows from them in interaction) are specified in Section 3.3 and applied throughout Section 4.

To examine dementia care communication in this vein, the analysis is based on a case study of naturally occurring interactions between a home caregiver and an older woman living with dementia in Taiwan. The focus is on how refusals are collaboratively managed and on the sequential environments in which different communication practices are deployed. This study is informed by gerontological sociolinguistics (Chen, 2024; Hamilton, 2008) and approaches dementia as a lived experience shaped through language, relationships, and the domestic settings in which long-term home care is delivered.

2. Literature review

2.1. Factors explaining refusals of dementia care

Refusals of care can be distressing for both care users with dementia and their caregivers (Fauth et al., 2016; Featherstone et al., 2019). Prior research links these refusals to a range of physiological, psychological, and environmental factors. Symptoms such as depression, hallucinations, and delusional thinking are associated with higher levels of care resistance (Galindo-Garre et al., 2015; Kang et al., 2021). Greater agitation and reduced independence in daily activities also correlate with a higher likelihood of refusal. Environmental triggers, including noise, discomfort, or overstimulation, may further contribute to care-resistant behaviour (e.g., reluctance, noncooperation, or refusal of assistance) reported in dementia care

contexts (Rader et al., 2006). In addition, some studies suggest that caregivers' communication styles may influence care users' acceptance, reluctance, or resistance during care delivery (Regier et al., 2019; Belzil and Vézina, 2015).

These studies offer valuable insights, but they are largely situated within medical and health-related frameworks, which tend to treat refusals as clinical symptoms or behavioural reactions to external stressors. In such accounts, communication is often treated as a background variable, secondary to physiological or environmental explanations. In contrast, this study treats communication as central to refusals of dementia care, an area that warrants closer academic attention. Refusals are analysed here as socially and interactionally situated practices. Rather than viewing them merely as indicators of distress or dysfunction, refusals are approached as interactional events that participants treat as such and that shape the subsequent organisation of care. This perspective makes it possible to examine how refusal episodes are managed through the caregiver's turn-by-turn responses (e.g. pursuits, redesigns, and alternative proposals) within the relational and spatial dynamics of Taiwan's long-term home-based dementia care.

The following section reviews recent work that examines how specific conversational strategies may be used to manage or reduce refusals in dementia care settings.

2.2. *Communication approaches to refusal management in dementia care*

Within communication-based approaches to dementia care (cf. Backhouse, 2021), person-centred care has become one of the most influential frameworks. Originating from the work of Tom Kitwood (1997), this approach emphasises recognising individuals with dementia as persons first by valuing their identity, agency, and lived experiences, rather than treating dementia primarily in terms of deficits and associated problems.

To operationalise this principle, several scholars identify communication features closely aligned with person-centred communication (Mundadan et al., 2023; Savundranayagam et al., 2007; Savundranayagam and Moore-Nielsen, 2015). These include facilitation (supporting and sustaining interaction), recognition (using names and referencing personal histories), negotiation (consulting preferences), and validation (acknowledging emotional states).

Many studies document the benefits of person-centred communication for staff training, relationship building, and quality of care (e.g., Backhouse, 2021; Terada et al., 2013; Tay et al., 2018), but they typically offer broad guidance. They often stop short of showing how specific practices operate turn by turn in real-time interaction, particularly when refusals of care arise as an interactional problem to be managed.

A smaller body of work addresses refusal management through communication more directly. Williams et al. (2004, 2009) investigated elderspeak, a speech style associated with exaggerated intonation, simplified grammar, and slowed pacing (Kemper, 1994), and showed that it can increase care resistance among nursing home residents with dementia. Their intervention work further suggests that reducing elderspeak can measurably lower instances of refusal (Williams et al., 2017), indicating that practices treated as nurturing may nonetheless undermine dignity and invite resistance. Christenson et al. (2011) examined command-giving practices in U.S. nursing homes and reported that alpha commands, which are direct, specific, and clearly actionable, were more successful in reducing refusals than beta commands, which are vague, interrupted, or impractical. These findings underscore how directive design can shape uptake in dementia-care encounters.

From a conversation analytic perspective, a UK-based study by O'Brien et al. (2020) examined refusal management in acute hospital interactions involving healthcare professionals and people living with dementia. The authors showed that both higher-entitlement request formats (treating an action as routine or expected) and lower-contingency designs (reducing the sense that compliance is optional) can promote cooperation. They reconceptualised refusals not as behavioural symptoms but as interactionally produced outcomes sensitive to how requests are designed and pursued, while also noting that heightened authority and entitlement must be balanced with attention to imposition and personhood.

To situate these dementia-care findings within a broader interactional literature, ethnomethodology and conversation analysis (EM/CA) have long examined how directive actions such as requests and instructions are organised as first pair parts that make particular forms of next action conditionally relevant (Schegloff, 2007). A core finding is that recipients' readiness to treat a requested action as due is shaped by turn design, especially how speakers display entitlement and contingency (Curl and Drew, 2008). Related work shows that resistance and non-acceptance are interactionally organised outcomes that can be displayed in diverse ways and managed through systematic next actions (Davidson, 1984; Flint and Rhys, 2023; Kent, 2012). Across settings, non-acceptance is commonly treated as a dispreferred response, often accompanied by delays, accounts, and reformulations, and it frequently provides the sequential environment for pursuits (such as re-issuing the directive) or redesigns (such as offering a modified alternative) oriented to restoring progressivity (Hao et al., 2022; Krummheuer, 2021; Lindström and Weatherall, 2015). Recent work further documents that, in cognitively vulnerable populations, resistance may be displayed through practices such as repair initiation, rejecting premises, topic shift, overt refusal, silence, laughter, and minimal responses (Ma and Huang, 2025). These EM/CA findings specify interactional mechanisms through which compliance and resistance emerge as sequential outcomes of action design and next-turn management in real-time care encounters.

Both EM/CA research on directive sequences and dementia-care studies converge on a central point: the design and pursuit of directive actions shape trajectories of uptake, including compliance, resistance, and subsequent reformulation or activity shifts. In dementia care, specific strategies are recommended, such as reducing elderspeak, improving directive clarity, and calibrating entitlement and contingency to support cooperation while safeguarding personhood and autonomy.

2.3. Research gaps and study rationales

As argued earlier, care refusals in dementia can be mitigated through careful communication design. Most communication-focused studies on refusal management, however, are situated in institutional settings such as nursing homes and hospitals and are largely grounded in Western care models. It therefore remains unclear how these interactional dynamics unfold in non-institutional environments. This study examines how refusals are managed communicatively within the specific context of Taiwan's long-term home care system.

In Taiwan's long-term home care, caregivers often operate without institutional support and must negotiate care tasks within care recipients' private space, where power dynamics can shift significantly. Taiwan also differs culturally from Western societies due to the influence of Confucian values that promote filial respect, face maintenance, and deference toward older adults. These values manifest in dementia-care interaction, shaping how Taiwanese caregivers navigate resistance compared to their Western counterparts. Such culturally specific communicative dimensions remain largely underexplored in the existing literature.

Prior research on person-centred care has also tended to prioritise general principles or macro-level training outcomes, leaving limited insight into how real-time refusals are addressed in everyday conversation (cf. O'Brien et al., 2020). Building on these gaps, the present study uses conversation analysis to examine naturally occurring interactions between a Taiwanese long-term home caregiver and an older woman living with dementia, recorded during routine care sessions with minimal researcher intervention.

Based on this aim, the following research questions are proposed to guide the conversation analysis in this study.

RQ1: What interactional practices does the caregiver employ to manage refusals, and how does the care recipient orient to these practices in subsequent turns?

RQ2: Which interactional practices appear to facilitate or impede eventual task acceptance, and what do these trajectories reveal about the interactional success of refusal management within this specific care context?

3. Methodology

3.1. The use of conversation analysis

Conversation Analysis (CA), developed in the 1960s by Harvey Sacks, Emanuel Schegloff, and Gail Jefferson, investigates how participants accomplish social actions through the sequential organisation of talk (Goodwin and Heritage, 1990; Hoey and Kendrick, 2017). Attending to the orderly, collaborative nature of interaction, CA treats conversation as jointly produced and interpreted by participants turn by turn in context (Psathas, 1994; Seedhouse, 2005). It examines practices such as turn-taking, repair, and action design, as well as how sequences are expanded, pursued, revised, or brought to closure (Gardner, 2004; Hutchby and Wooffitt, 2008; Liddicoat, 2021; Wooffitt, 2005). CA has also been applied to dementia care interaction in medical, clinical, and institutional settings, showing how care tasks and personhood-related concerns are managed *in situ* (Kindell et al., 2017; Mazeland, 2005).

This study employs CA to examine how a Taiwanese long-term home caregiver selects and adapts interactional practices in response to refusals from a care recipient living with dementia. The analysis focuses on the sequential organisation of these episodes, including initial actions, caregiver responses, and subsequent interactional adjustments. Refusals are treated not as fixed behavioural symptoms of dementia, but as sequentially organised actions embedded in the lived reality of dementia care. This micro-level focus on conversational sequences provides insight into how both parties interactionally manage resistance or facilitate task acceptance.

3.2. Data and participants

The data for this study were collected through ethnographic fieldwork, enabling observation of participants in a naturalistic home-care setting and the documentation of interactional conduct in context (Bailey et al., 2015; Hammersley and Atkinson, 1995; Roberts, 2009; Rampton, 2009). Ethnography informed the data collection and contextualisation (non-participant observation and field notes), while Conversation Analysis (CA) served as the primary analytic approach for examining the sequential organisation of refusal episodes in recorded talk.

Fieldwork consisted of non-participant observation, real-time field notes, and audio recordings of routine care encounters between a Taiwanese long-term home caregiver and an older woman (aged 85) living with moderate-stage vascular dementia. The care recipient retains clear memories of family members but has difficulty recalling neighbours and frequently repeats questions. She receives home-care visits three times a week, primarily for scheduled outdoor activities, with each session lasting approximately 1.5 h. She uses a wheelchair for mobility but occasionally uses a walker for short distances. Given her largely sedentary routine outside scheduled care sessions, the caregiver regularly encourages walking to help maintain mobility. At the time of data collection, the caregiver had worked with the care recipient for three years.

To minimise researcher influence, the second author was present during data collection but remained physically distant and did not intervene in the care process. Field notes documented relevant contextual circumstances and observable

conduct not available in the audio record. Because the recordings are audio-only, the analysis is grounded primarily in participants' talk and its sequential organisation; any reference to non-verbal conduct is restricted to what was directly observed and noted at the time of recording.

Data were collected between 10 July and 8 September 2023, resulting in 26 audio files totalling 2340 min of recorded interaction (approximately 90 min per session on average). From this corpus, segments involving refusals of care were identified for detailed analysis. The selection of the four extracts presented in Section 4 was guided by two primary criteria.

First, each extract contains an analytically tractable refusal sequence with sufficient surrounding turns to demonstrate how the caregiver's subsequent conduct is shaped by the prior refusal. Second, each extract illustrates a recurrent practice observed across the collection (e.g., deferring the due time, invoking third-party accountability, or using incrementally downgraded proposals). These extracts thus function as collection-based exemplars of the distinct, recurring ways resistance is managed in this specific home-care case. All extracts are presented in the original Chinese followed by an English translation, with transcription symbols following Jeffersonian conventions (Jefferson, 1985; see Appendix A).

3.3. Analytic procedure

In line with Conversation Analysis, caregiving directive actions are treated as first-pair parts (or first-position actions) that make some recipient uptake conditionally relevant next (e.g., acceptance or compliance, an account, a counter-proposal, or another relevant response). Offers, proposals, requests, and instructions are treated as subtypes of directive actions and are identified through their turn design and sequential placement; that is, how they project a specific next action and thereby shape the unfolding care trajectory. Instruction-instructed action relations are treated as sequences of two contingent actions, in which the instructed action may be oriented to as due immediately or made relevant for a later moment in the encounter (De Stefani, 2018). The analysis therefore tracks not only the production of a directive, but also how the timing of compliance is organised within the ongoing activity.

Refusals are treated as second-position responses (or response-relevant non-uptake) that display non-acceptance of a prior directive action. In the present data, refusals are identified through recurrent formats and their sequential consequences, including explicit negation (e.g., “不要/no”), delaying or deferring moves (e.g., “晚一點/later”), complaints that treat pursuit as pressuring (e.g., “給我趕/you're rushing me”), alternative proposals that redirect the trajectory, and silences or gaps in positions where a response would ordinarily be due. These non-acceptance displays are analysed in terms of the actions they make relevant next, such as pursuit, redesign, downgrade, deferral, reframing, or activity shift.

The recordings were reviewed to locate refusal-relevant episodes, namely, segments where a caregiving directive action was met with a dispreferred response. These episodes were then transcribed in detail using Jeffersonian conventions (see Appendix A), including timing (pauses and gaps), salient prosodic features, and relevant contextual notes. Through repeated, turn-by-turn inspection, the analysis traced the trajectory of the interaction: from the composition of the initial directive and the recipient's formatted non-acceptance to the caregiver's management of subsequent turns. Particular attention was paid to how the recipient's next-turn uptake displayed either alignment or continued resistance.

4. Findings

Section 4 presents four CA extracts that illustrate recurrent ways in which refusals are managed in the examined Taiwanese long-term home care conversations. Following an excerpt–commentary unit format (Emerson et al., 1995), each extract is presented in full before the analytic commentary. In each extract, a caregiving action (offer, proposal, request, or instruction) makes a preferred next action conditionally relevant, and the care recipient displays non-acceptance through varied turn formats, including explicit negation, delaying moves, alternative proposals, or non-uptake. The analyses track how these refusal displays shape what becomes relevant next for the caregiver, including pursuit, redesign, or deferral (i.e., rescheduling the activity), and how alignment or continued resistance is observed in the care recipient's subsequent uptake. The extracts show refusal management as a dynamic sequential accomplishment produced turn-by-turn, rather than a fixed individual trait or a clinical “symptom” of dementia.

Extract 1.

Extract 1 occurs during a breakfast routine involving A (the care recipient) and B (the caregiver). The interaction begins with B initiating a proposal for A to consume a nutritious drink. Following this initiation, A provides an immediate refusal, which leads to a brief negotiation over the timing of the task.

- 01 B: 先喝一下這個((營養飲)),好不好?
Have a bit of this ((nutritious drink)) first, okay?
- 02 A: 不要
I don't want it
- 03 B: 不要緊 = 我們>等一下回來<再喝啊(2.0)你這陣子<胃口較不好欸> = 天氣比較熱嗎?
It's okay = we can have it > when we come back < later (2.0) your < appetite hasn't been so good > lately = Is it because the weather is a bit hot?
- 04 A: °早上吃不太下°
°I can't really eat in the morning°
- 05 B: 可能天氣也有關係。
Maybe it's because of the weather.

- (2.0)
 06 B: 我先幫你量((血壓)) = 好不好?
 Let me check ((your blood pressure)) first = okay?
 (1.2) ((B checks A's blood pressure.))
 07 B: 又再流鼻水了(.)我們來曬太陽 = > 太陽曬一曬 < 就好了(2.0)你這禮拜起來都這樣?
 Your nose is running again (.) Let's get some sunshine = > a bit of sun < will do you good (2.0) has it been like this all week?
 (7.0) ((B continues checking A's blood pressure.))
 08 B: 阿姨來(.) 飯後的(2.0)> 吃一吃, 快來曬太陽 ≤ 曬一曬你就好了。
 Auntie come (.) the after-meal one (2.0) > take it, then come get some sunshine ≤ a bit of sun and you'll feel better.
 09 A: 今天我的保鏢在哪咧? [Laugh]
 Where's my bodyguard today? [Laugh]
 10 B: 有啦 = 就我(.)在你後面。
 Yes = it's me (.) behind you.
 11 A: 保鏢[laugh]
 Bodyguard [laugh]
 12 B: 嘿阿[laugh](.) 阿姨, 也要走一走(2.0)阿姨, > 吃一吃 ≤ 我們也來去((公園)).
 Yeah [laugh](.) auntie, you should also walk a bit (2.0) auntie, > take it ≤ and we'll go ((to the park)) too.
 ((They stand up and leave to go outside.))

Extract 1 shows how B manages A's rejection of a caregiving proposal by deferring the activity and soliciting a candidate account in order to maintain alignment. A then initiates a playful shift in footing, which B takes up. This produces an affiliative context in which B can link two potentially resistant care routines, medication-taking and going to the park, within a single ongoing trajectory. In doing so, B reduces the sequential space available for further rejection and sustains progressivity as the encounter moves into embodied activity.

Extract 1 opens with an offer–rejection adjacency pair (lines 01–02). In line 01, B produces an offer with a tag-like solicitation, “先喝一下這個((營養飲)), 好不好?/Have a bit of this ((nutritious drink)) first, okay?”, making acceptance conditionally relevant. In line 02, A responds with a bald negation, “不要/I don't want it”, producing a direct second pair part (SPP) that blocks the projected completion of the sequence. However, the caregiver's next turn (line 03) treats this rejection as non-terminal by deferring the relevance of the instructed action, drinking, to a later point in the activity trajectory. This postpones immediate compliance without abandoning the proposed action, and keeps the task structurally available within the ongoing care routine (De Stefani, 2018; De Stefani and Gazin, 2014). In the same turn, B initiates an account trajectory by offering a candidate explanation for A's resistance, “你這陣子<胃口較不好欸>/your < appetite hasn't been so good > lately”, and shifting attention to an environmental factor, “天氣比較熱嗎?/is it because the weather is a bit hot?” (line 03). The interactional work shifts here from pursuing compliance to establishing an intelligible basis for non-acceptance.

Lines 04–05 develop the reasoning. The care recipient offers a low-volume account (line 04), which the caregiver then ratifies and extends (line 05). At this point, the nutrition task is set aside rather than dropped, and progressivity is maintained through empathic alignment.

After a 2.0-s gap (line 05), B initiates a stepwise move into an alternative care activity: “我先幫你量((血壓)) = 好不好?/Let me check ((your blood pressure)) first = okay?” (line 06). The “我先/first” design, together with the tag format, opens participation on a new trajectory. As the measurement proceeds, B continues the health-assessment line, “又再流鼻水了/your nose is running again”, and proposes a further course of action, “我們來曬太陽/let's get some sunshine”, packaged as a light remedy, “太陽曬一曬<就好了>/>a bit of sun < will do you good” (line 07).

In line 08, B produces a directive package that links a new task, medication-taking, to the sunshine trajectory: “阿姨來(.) 飯後的(2.0)> 吃一吃, 快來曬太陽 ≤ 曬一曬你就好了/Auntie come (.) the after-meal one (2.0) > take it, then come get some sunshine ≤ a bit of sun and you'll feel better”. The initial summons, “阿姨來/Auntie, come”, works as a pre-directive that builds sequential momentum. As an invitation-like summons, it also invokes a preference for alignment (Davidson, 1984; Drew, 1984). The medication directive, “吃一吃/take it”, is embedded within a beneficial activity frame, “曬太陽/get some sunshine”, which advances the care project without relying on a standalone high-deontic command. By tying medication-taking to the sunshine invitation, B makes non-alignment more interactionally costly, since rejection would now require an account that halts the ongoing progressivity of the care routine.

In the next turn, the care recipient shifts into a playful frame by initiating a joke: “今天我的保鏢在哪咧?/Where's my bodyguard today?” (line 09). B takes up this affiliative bid (Stivers, 2008; Stivers et al., 2011) immediately (line 10), and the shared laughter (lines 09, 11, 12) neutralises any interactional tension from the earlier rejection. This shift in footing (Goffman, 1981) provides a context in which B can return to the medication directive under conditions of stronger alignment.

In line 12, B reorients to the next embodied step: “阿姨,> 吃一吃 ≤ 我們也來去((公園))/auntie, > take it ≤ and we'll go ((to the park)) too”. The latching between the directive, “take it”, and the proposal to go to the park compresses the space in which a counter-move could be launched. The participants' subsequent joint departure displays the successful negotiation of the care trajectory and a restoration of interactional progressivity.

Extract 2.

Extract 2 takes place after B has assisted A in moving from her bedroom to the living room. Upon arrival, A expresses a desire to remain seated (line 01), while B introduces a plan for a walk to a nearby park to visit with friends (the “aunties” mentioned in line 02). B's proposal involves using A's wheelchair, suggesting that the act of sitting can be maintained during the transition outdoors.

- 01 A: 先坐一下。
Let me sit for a bit first.
(1.0)
- 02 B: 你那台((A的輪椅)) = 你也能坐啊 = 我>推你去<公園:跟阿姨們聊天啊:
That one ((A's wheelchair)) = you can sit on that too = I'll > push you < to the park::so you can chat with the aunties::
(2.0)
- 03 A: 不用啦。
No need.
(3.0)
- 04 B: 你也能坐著啊 = 你也坐椅子啊(2.0)來,快起來
You can still sit = you can sit on a chair too (2.0) come, get up now
- 05 A: 給我趕
You're rushing me.
(3.0)
- 06 B: 當然啊 = 不然等等阿姨問你怎麼還沒出來 = 對吧?
Of course I am = otherwise the aunties will ask why you haven't come out yet = right?
(3.0) ((A stands up from the chair and starts to move))
- 07 B: 來 = 你今天怎麼那麼勇! = 自己走這樣 = 一直走過去::
Come on = why are you so **STRONG** today! = walking by yourself like this = going all the way over there:
(14.0) ((A looks on while B continues the activity trajectory.))
- 08 B: 哇! = 今天這台小推車((A的行走輔助工具))沒推來= >你就一直<走來!
WOW! = today I didn't even bring this little cart ((A's walking aid)) =>and you just kept < walking over!
((A does not respond.))

Extract 2 demonstrates how non-compliance is managed through a series of action redesigns that steadily escalate the deontic pressure. As the sequence unfolds, B addresses A's shifting refusals by first framing the activity as compatible with A's preferences, then upgrading the proposal to a direct imperative, and finally invoking third-party accountability. Although the interaction concludes with intensified praise, it fails to secure verbal uptake, leaving B's assessments unrati- fied by the care recipient.

The interaction in Extract 2 begins with A's delaying move in line 01 (先坐一下/“let me sit for a bit first”), which differentially holds off the projected park-going trajectory without a bald rejection. B responds in line 02 with a compatibility reframe, designing the outing as sitting-compatible (你那台 ... 你也能坐啊/“that one ... you can sit on that too”) and adding a social rationale (跟阿姨們聊天/“chat with the aunties”). By incorporating A's preference into the care agenda, B attempts to bridge the misalignment. After a 2.0-s gap, however, A produces an explicit non-acceptance in line 03 (不用啦/“no need”), which provides the sequential basis for B's next escalation.

In line 04, B repeats the compatibility frame and then produces an upgraded pursuit: “來, 快起來” (“come, get up now”). Here, 來 (“come”) functions as a summons-to-action (see also Extract 1), while the imperative “快起來” (“get up now”) upgrades the activity from a negotiable proposal to a demand for immediate embodied compliance. A treats this shift as coercive; in line 05, she produces a complaint (給我趕/“you're rushing me”), which frames B's turn (line 04) as an accountable over-pursuit. After a 3.0-s gap, B responds in line 06 by ratifying the rushing characterisation (當然啊/“of course I am”) and invoking third-party accountability (不然等等阿姨問你 .../“otherwise the aunties will ask ...”). This move increases the normative weight of task by making A's refusal socially reportable. This escalation successfully secures the change in activity, as A immediately stands and begins to move.

Once A is in motion, B produces an intensified praise sequence in lines 07 and 08. In line 07, B uses an upgraded assessment (那麼勇/“so **STRONG**”), with emphatic stress to evaluate A's movement. In line 08, this is followed by a surprise-marked assessment (哇!/“**WOW!**”) and a diminutive formulation (小推車/“little cart”). These turns treat A's ordinary walking as notably exceptional. By framing a routine mobility task as a remarkable feat, B's turn design in lines 07 and 08 displays features associated with patronising speech or elderspeak (Williams et al., 2017). This intensified evaluative effort fails to secure verbal uptake, since the care recipient provides no acknowledgment token in the relevant next position, leaving B's assessments unrati- fied.

Extract 3.

Extract 3 involves a pre-scheduled hair salon appointment for A that had been arranged by her family. The interaction begins with B initiating the outing by focussing on the preparatory task of fitting A with a back brace. The sequence that follows centres on a negotiation between A and B regarding the timing of the departure and the eventual destination of the outing.

- 01 B: 這個((護腰))帶去 = 坐那((在髮廊)) = 這樣腰才有得靠
Take this ((back brace)) with you = when you sit there ((at the salon)) = so your back can have support.
(3.0)
- 02 A: 晚一點再去。
Go a bit later.
(2.0)
- 03 B: 要多晚?
How much later?
(1.0)
- 04 A: 較晚

- Later.
(2.0)
05 B: 我先來看((髮廊))有沒有人
I'll first go check ((the salon)) to see if there's anyone there.
06 A: 不用
No need.
07 B: 好啊(2.7)要不你要先去公園
Alright then (2.7) how about you go to the park first?
08 A: 嗯 =
Mm-hm =
09 B: = 好(.)走吧
= Okay (.) let's go

Extract 3 demonstrates how the caregiver (B) manages a scheduled care trajectory concerning a visit to a hair salon in the face of the recipient's (A) shifting formats of resistance. The sequence is organised around a stepwise action redesign, in which B moves from an instruction-like initiation in line 01 to a pursued specification in line 03 following a deferral, and finally to a sequential reorientation in line 07 after an explicit rejection. A's resistance evolves from a delaying format that leaves the task possible to a bald negation that effectively forecloses the original trajectory. Throughout the sequence, silences at transition-relevant points following B's initiations in lines 01, 03, and 07 create sequential environments that index a lack of alignment from the care recipient. These conspicuous absences prompt B's subsequent reformulations, which are consistently oriented towards maintaining progressivity in the care task.

The sequence unfolds across three reformulation cycles, each occasioned by a distinct display of resistance from A. First B's opening turn in line 01 offers a back brace alongside an account: “這個((護腰))帶去 = 坐那((在髮廊)) = 這樣腰才有得靠” (“Take this back brace with you, when you sit there at the salon, so your back can have support”). This turn projects the salon visit as the relevant trajectory, frames the instruction as care-oriented, and makes compliance the conditionally relevant next action. A 3.0-s gap follows, which B treats as a noticeable absence of alignment. This disalignment is further displayed when A eventually responds with a deferral: “晚一點再去” (“go a bit later”) in line 02. The deferral withholds immediate uptake while keeping the salon project available for further negotiation.

B responds by pursuing specification: “要多晚?” (“how much later?”) in line 03. A's next turn, “較晚” (“later”) in line 04, remains non-specific, withholding the requested information while keeping the sequence moving. Here, resistance is handled as a negotiation of timing rather than a definitive rejection. Following A's non-specific response, a 2.0-s gap emerges. During this silence, A does not elaborate, effectively suspending the progressivity of the salon-going trajectory. B treats this absence of uptake as grounds for an action redesign.

Therefore, in line 05, B attempts to maintain the trajectory by redesigning the next step as a preparatory action she can perform independently: “我先來看((髮廊))有沒有人” (“I'll first go check the salon to see if there is anyone there”). This reformulation shifts the deontic load away from A's immediate participation, redistributing agency so that B absorbs the first actionable step. A, however, blocks this redesigned action with a bald negation: “不用” (“no need”) in line 06. Delivered without an account, this response is a marked dispreferred that upgrades the resistance relative to the earlier deferrals. It closes off not only the original proposal but also B's redesigned version of it.

B's subsequent turn, “好啊” (“alright then”) in line 07, receipts A's rejection without contest. Rather than functioning as a fully-fledged sequence-closing third (Schegloff, 2007), it operates as a pro-forma acknowledgment in which the closing function and a pivot toward redesign are collapsed into a single token. A 2.7-s gap follows at a transition-relevance place, during which B's next action is momentarily withheld. B then treats this silence as a structural pivot (Heritage, 1984) and initiates a sequential reorientation through an alternative proposal: “要不你要先去公園” (“how about you go to the park first?”). The “要不” (“how about”) preface re-opens a participation opportunity and pursues progressivity through a different activity.

A produces a minimal response, “嗯 = ” (“mm-hm =”) in line 08, which B immediately latches with an acceptance and a proposal for joint action: “ = 好(.)走吧” (“ = okay (.) let's go”) in line 09. Rather than waiting for a fuller or more explicit uptake, B treats this minimal affirmation as interactionally sufficient to close the proposal sequence. The latching itself enacts progressivity. B's turn arrives without any gap, performing urgency and forward momentum.

Extract 4.

In this extract, B (the caregiver) attempts to facilitate a mobility exercise with A (the care recipient) using a walking aid provided by A's daughter. Despite B's prompts to incorporate the device into A's dementia care activities, A resists the suggestion, leading to a negotiation of physical tasks.

- 01 B: 我們(1.0)用這台((行走輔助器)) = <慢慢地>推到這就好了
We'll (1.0) use this ((walking aid)) = and <slowly> push it over to just here
02 A: 不要啦 = 坐在這看電視
No = I'll just sit here and watch TV.
03 B: 電視又沒什麼好看 = 我們站這就好了
There's nothing good on TV = we can just stand here
(4.7)
04 B: 門口這就好了
Just by the doorway is fine

- 05 A: 不用 = 在這坐。
No need = I'll sit here.
- 06 B: 你走過去 = 我把你扶緊緊
You walk over there = I'll hold you tight
- 07 A: 不用。
No need.
- 08 B: 要走幾步 = 你的腳才不會軟
You should walk a few steps = so your legs won't get weak
(52.3)
- 09 B: 阿姨 = 不然, 我們來做手部運動 ((B伸展A的手並握著運動))
Auntie = otherwise, let's do some hand exercises instead
((B holds A's hands and stretches them for exercise.))
(1.0)
- 10 A: 嗯。
Mm.
(6.0)
((A follows B's movements.))
- 11 B: ((B holds A's hands and stretches them for exercise))對對對 = 這樣血路才會通
yes yes yes = this way your blood circulation will flow

Extract 4 illustrates the management of persistent resistance through stepwise redesign and an eventual trajectory shift. In this sequence, A's bald negations and response-relevant non-uptake (silence) provide the sequential environment for B's next turns. B first responds with further downgraded renewals following shorter gaps, and later shifts to an alternative activity following a prolonged silence.

The sequence begins with B's directive proposal in line 01: "... 用這台((行走輔助器)) ... 推到這就好了"/"... use this ((walking aid)) ... push it over to just here". Several design features frame the task as low-demand: a distal pointer ("到這"/"to here"), a minimiser ("就好了"/"just"), and slowed prosodic delivery ("<慢慢地>"). In the adjacent turn (line 02), A produces a dispreferred response in the form of a bald negation ("不要啦"/"no"), immediately followed by an incompatible alternative trajectory, "坐在這看電視"/"sit here and watch TV". By projecting a competing course of action, A does not only decline the task but actively resists B's mobility-oriented agenda.

In line 03, B treats A's rejection as grounds for redesign rather than as a reason to abandon the trajectory. B first counters A's TV account and then produces a downgraded mobility proposal, "我們站這就好了"/"we can just stand here". By shifting the requirement from walking to standing, B reduces the deontic load of the care task while keeping the mobility trajectory in play, and pursues a smaller, easier embodied move (see [Curl and Drew, 2008](#)).

A 4.7-s gap follows line 03. Within CA, this silence occurs in a response-relevant position where uptake would ordinarily be due. B orients to the lack of progression in line 04 by narrowing the pursuit, "門口這就好了"/"just by the doorway is fine". This turn further restricts the spatial scope of the request and treats the standing or moving trajectory as still relevant.

A's next turn in line 05, "不用 = 在這坐"/"no need = I'll sit here", reasserts the competing sitting trajectory. B's response in line 06 makes a further redesign attempt by adding an assistance component, "我把你扶緊緊"/"I'll hold you tight". A's immediate block in line 07, "不用"/"no need", displays continued non-acceptance across successive versions of the mobility proposal.

In response to the recipient's continued non-acceptance, B produces a multi-unit turn in line 08 that pursues compliance through an account-based redesign. Within this turn, the mobility task is framed as both minimal and preventative: the inclusion of the minimiser "a few steps" ("幾步") reduces the deontic load of the directive by presenting the physical requirement as a low-effort action, while the warrant "so your legs won't get weak" ("你的腳才不會軟") grounds the pursuit in an epistemic rationale tied to health relevance. By anchoring the directive in this preventative logic rather than a standalone command, B attempts to justify the necessity of the task while simultaneously lowering the participation threshold for the recipient.

After this warrant, an extended 52.3-s gap follows. B treats the continued absence of progression as closing the walking exercise and initiates an explicit trajectory shift in line 09. The proposal is prefaced with the connective "不然"/"otherwise", which operates as a sequential pivot. In using this marker, B displays orientation to the breakdown in progressivity of the prior mobility pursuit and redirects the encounter to an alternative activity track. A's minimal receipt, "嗯"/"mm" (line 10), displays alignment with the shift, and her immediate transition into the hand exercises further supports this. B then ratifies the transition with the multiple saying "對對對"/"yes yes yes", which endorses and stabilises the new activity. In this way, silence in Extract 4 operates as an interactional mechanism of non-progression that conditions B's move from persistent pursuit to a more radical reformulation.

5. Discussion and conclusion

5.1. Refusal management as sequential practice

This study shows that, in Taiwanese long-term home care, refusals are not interactional 'dead ends' but pivot points that occasion further sequential adjustment. Across the extracts, caregiver directive actions (offers, proposals, and instructions) make acceptance conditionally relevant as the preferred second pair part. When the care recipient displays non-acceptance,

the activity is not simply blocked. Instead, non-acceptance creates a specific sequential environment that the caregiver treats as a site for turn redesign. The analysis demonstrates that refusal management is not applied uniformly but is systematically organised in response to the specific format of the care recipient's non-acceptance.

First, when a refusal is delivered as an explicit negation (e.g. "I don't want it", Extract 1), the caregiver treats the rejection as closing the immediate sequence while keeping the wider activity trajectory intact. By accepting the negation in the local slot, the caregiver avoids escalated pursuit and preserves the possibility of re-initiating the task later, in a more favourable sequential environment.

Second, when resistance is produced as a delaying or deferring move (e.g. "go a bit later", Extract 3), it is treated not as a definitive rejection but as a negotiable baseline that maintains the task's relevance. This format makes a specification sequence relevant, in which the caregiver uses follow-up questioning and preparatory reformulations to bridge the recipient's proposed delay and the practical requirements of the care routine. In these instances, the caregiver may redesign the next step as a preparatory action she can perform independently, effectively absorbing the initial agency to sustain the trajectory even while the recipient's participation is deferred.

Thirdly, pauses and response gaps are analysed as a form of passive non-acceptance. When silence occurs in a response-relevant position where uptake is conditionally due, the caregiver treats it as non-uptake rather than as a space for internal 'cognitive processing'. This grounds the function of silence in its sequential placement as an interactional mechanism of non-progression. Extract 4 illustrates this explicitly: the extended 52.3-s gap is treated as a sequence-terminating event, followed not by a repetition of the failed directive, but by a radical trajectory shift, prefaced with the sequential pivot "otherwise" ("不然"), towards an alternative activity with lower immediate demands. In this sequential environment, silence functions much like explicit negation; both signal a blockage in the current activity that the caregiver manages through a redesigned, lower-stakes participation slot. By working with the recipient's displayed stance to maintain progressivity, the caregiver enacts a person-centred approach anchored in observable interactional conduct.

Finally, when resistance is framed as a complaint about the caregiver's conduct, as seen in the care recipient's turn "You're rushing me" in Extract 2, the interactional focus shifts from the care task to relational tension. In these instances, the caregiver treats the resistance as non-terminating and uses justifications to re-legitimise the directive and secure physical compliance. Nevertheless, this study suggests that such persistence often carries significant interactional costs. When task progressivity is achieved through high-pitched, exaggerated praise or diminutive forms, both of which are features characteristic of elderspeak (Williams et al., 2017), the turn design frequently fails to elicit affiliative uptake from the recipient. The resulting extended silence provides an empirical indicator of an unratified sequence (see Extract 2). These cases highlight a central tension in dementia care, where justificatory persistence and patronising speech styles may successfully advance a care trajectory while simultaneously eroding rapport and limiting the achievement of joint alignment.

5.2. Sociocultural implications as a secondary interpretive layer

The analysis in Section 4 is grounded strictly in what participants make visible through their turn-by-turn conduct. In this section, we step back from that sequential level to consider what those interactional patterns might tell us about the broader cultural context in which they occur, drawing cautiously on Bucholtz and Hall's (2005) sociocultural linguistic framework. We do so only where the participants' own linguistic choices and orientations point towards recognisable features of Taiwanese relational practice, rather than invoking cultural values as a ready-made explanatory resource.

One interactionally visible resource is relational address. The caregiver's repeated use of "阿姨/Auntie" positions the care recipient within a kinship-like frame that is recognisable in Taiwanese interaction. This address term performs specific interactional work by organising the participation framework and fitting directive and proposal actions into a familiar relational format. In the Taiwanese context, this choice of address invokes Confucian-based expectations of filial respect, effectively situating the professional dementia care interactions within a culturally familiar relational practice. Such practice stands in contrast to the formal or clinical address patterns typical of Western institutional care, where names and titles usually establish a more standardised professional distance.

Furthermore, the recurrent use of design features such as deferrals, downgraded directive renewals, and task trajectory shifts can be interpreted as stance displays that prioritise non-confrontation and face maintenance. Rather than asserting high-deontic authority, the caregiver often employs these strategies to navigate resistance while preserving the care recipient's dignity and the relational bond. Such practices index locally meaningful values, such as relational smoothness and sustained facework, as evidenced through the participants' visible orientations and next-turn uptake.

5.3. Limitations and future directions

This study is based on a single dyad and a limited recording period, precluding claims of statistical generalisability. Instead, the focus is on analytic generalisation, demonstrating how specific sequential practices are deployed and the interactional consequences they produce in this home-care setting. Future research could expand this scope by examining a broader range of caregivers, recipients, and activities, including comparisons with institutional contexts.

Additionally, as the current dataset is audio-based, the analysis is restricted to vocal conduct. While contextual notes helped capture some embodied actions, incorporating video data in future studies would allow for a multimodal

investigation of resources such as gaze, gesture, touch, and spatial positioning. Such data would facilitate a more nuanced understanding of the timing of physical compliance relative to directives and pauses.

Finally, following these interactions over an extended period would reveal how caregivers become attuned to a dementia care recipient's specific patterns of resistance and learn to pre-emptively refine the design of their directives. This longitudinal perspective would also capture how these practices evolve alongside the progression of dementia, offering a clearer picture of how filial respect and the relational bond are sustained as cognitive and social circumstances change.

5.4. Conclusion

This Taiwan-based home-care case study demonstrates that refusal management in dementia care is achieved through sequential practices that treat resistance as a workable interactional event rather than a fixed behavioural symptom. The caregiver sustains task progressivity by redesigning next actions in response to explicit refusals and non-uptake, which involves deferring the timing of instructed actions, downgrading proposals, shifting tasks, and re-opening participation through alternative offers.

The analysis also clarifies the function of gaps and silences as sequential resources that mark non-uptake environments and create opportunities for redesigned next turns. Finally, a complementary sociocultural linguistic lens (Bucholtz and Hall, 2005) helps describe how relational address and stance displays index locally meaningful values, such as filial respect and face maintenance, which become consequential for how care is negotiated in home settings. Ultimately, these findings contribute to interactional research on dementia care in a Mandarin-speaking context, underscoring the importance of grounding claims in sequential evidence while remaining attentive to the social meanings participants make relevant in talk.

CRedit authorship contribution statement

Chin-Hui Chen: Writing – review & editing, Writing – original draft, Validation, Supervision, Methodology, Investigation, Formal analysis, Data curation, Conceptualization. **Chia-Yu Li:** Resources, Methodology, Investigation, Data curation.

Declaration of competing interest

Both of the authors, Chin-Hui Chen and Chia-Yu Li, have nothing to declare.

Appendix A. Transcription conventions

(0.0)	Timed pause in seconds (to one decimal place); smallest unit 0.1s
(.)	Micro-pause, hearable but shorter than 0.1s
=	Latching (no measurable gap between turns/TCUs)
::	Sound prolongation
word	Emphasis
WORD	Increased volume (louder)
^o word ^o	Reduced volume (softer)
> word <	Faster speech
< word >	Slower speech
?	Rising intonation
.	Falling intonation
,	Continuing (non-final) intonation
((...))	Field-notes/contextual information/non-verbal conduct relevant to the interaction (e.g., movement, handling objects, audible background events)
[Laugh]	Laughter
!	Exclamatory intonation (e.g., surprise/excitement)

References

- Backhouse, T., 2021. Understanding and reducing refusals of personal care in dementia. *Nurs. Resid. Care* 23 (3), 1–8.
- Bailey, S., Scales, K., Lloyd, J., Schneider, J., Jones, R., 2015. The emotional labour of health-care assistants in inpatient dementia care. *Ageing Soc.* 35 (2), 246–269.
- Belzil, G., Vézina, J., 2015. Impact of caregivers' behaviors on resistiveness to care and collaboration in persons with dementia in the context of hygienic care: an interactional perspective. *Int. Psychogeriatr.* 27 (11), 1861–1873.
- Bucholtz, M., Hall, K., 2005. Identity and interaction: a sociocultural linguistic approach. *Discourse Stud.* 7 (4–5), 585–614.
- Caputo, A., 2021. The emotional experience of caregiving in dementia: feelings of guilt and ambivalence underlying narratives of family caregivers. *Dementia* 20 (7), 2248–2260.
- Chen, C.-H., 2024. Long-term home care communication with older people in Taiwan: strategies and rationales. *Work. Older People* 28 (2), 149–160.
- Christenson, A.M., Buchanan, J.A., Houlihan, D., Wanzek, M., 2011. Command use and compliance in staff communication with elderly residents of long-term care facilities. *Behav. Ther.* 42 (1), 47–58.
- Curl, T.S., Drew, P., 2008. Contingency and action: a comparison of two forms of requesting. *Res. Lang. Soc. Interact.* 41 (2), 129–153.

- Davidson, J., 1984. Subsequent versions of invitations, offers, requests, and proposals dealing with potential or actual rejection. In: Atkinson, J.M., Heritage, J. (Eds.), *Structures of Social Action: Studies in Conversation Analysis*. Cambridge University Press, pp. 102–128.
- De Stefani, E., 2018. Formulating direction: navigational instructions in driving lessons. *Int. J. Appl. Ling.* 28 (2), 283–303.
- De Stefani, E., Gazin, A.-D., 2014. Instructional sequences in driving lessons: mobile participants and the temporal and sequential organization of actions. *J. Pragmat.* 65, 63–79.
- Drew, P., 1984. Speakers' reportings in invitation sequences. In: Atkinson, J.M., Heritage, J. (Eds.), *Structures of Social Action*. Cambridge University Press, pp. 129–151.
- Emerson, R.M., Fretz, R.I., Shaw, L.L., 1995. *Writing Ethnographic Fieldnotes*. University of Chicago Press.
- Fæø, S.E., Husebo, B.S., Bruvik, F.K., Tranvåg, O., 2019. "We live as good a life as we can, in the situation we're in"—the significance of the home as perceived by persons with dementia. *BMC Geriatr.* 19 (158), 1–10.
- Fauth, E.B., Femia, E.E., Zarit, S.H., 2016. Resistiveness to care during assistance with activities of daily living in non-institutionalized persons with dementia: associations with informal caregivers' stress and well-being. *Aging Ment. Health* 20 (9), 888–898.
- Featherstone, K., Northcott, A., Bridges, J., 2019. Routines of resistance: an ethnography of the care of people living with dementia in acute hospital wards and its consequences. *Int. J. Nurs. Stud.* 96, 53–60.
- Flint, N., Rhys, C.S., 2023. Teenage resistance to a parental threat: intercepting an action-in-progress as a form of resistance. *J. Lang. Soc. Psychol.* 42 (5–6), 610–629.
- Frisoni, G.B., Altomare, D., Ribaldi, F., Villain, N., Brayne, C., Mukadam, N., et al., 2023. Dementia prevention in memory clinics: recommendations from the European task force for brain health services. *Lancet Reg. Health—Europe* 26, 100576.
- Galindo-Garre, F., Volicer, L., van der Steen, J.T., 2015. Factors related to rejection of care and behaviors directed towards others: a longitudinal study in nursing home residents with dementia. *Dement. Geriatr. Cogn. Dis. Extra* 5 (1), 123–134.
- Gardner, R., 2004. Conversation analysis. In: Davies, A., Elder, C. (Eds.), *The Handbook of Applied Linguistics*. Blackwell Publishing, pp. 262–284.
- Gjelstad, A., Oksholm, T., Alvsvåg, H., Bruvik, F., 2023. Trust-building interventions to home-dwelling persons with dementia who resist care. *Nurs. Ethics* 30 (7–8), 975–989.
- Goffman, E., 1981. *Forms of Talk*. University of Pennsylvania Press.
- Goodwin, C., Heritage, J., 1990. Conversation analysis. *Annu. Rev. Anthropol.* 19, 283–307.
- Hamilton, H.E., 2008. Language and dementia: sociolinguistic aspect. *Annu. Rev. Appl. Ling.* 28, 91–110.
- Hammersley, M., Atkinson, P., 1995. *Ethnography: Principles in Practice*, second ed. Routledge.
- Hao, Q., Guo, H., Li, C., Yang, S., 2022. Subsequent actions engendered by the absence of an immediate response to the proposal in Mandarin mundane talk. *Front. Psychol.* 13, 942266.
- Henriksen, K., Joseph, A., Zayas-Cabán, T., 2009. The human factors of home health care: a conceptual model for examining safety and quality concerns. *J. Patient Saf.* 5 (4), 229–236.
- Heritage, J., 1984. A change-of-state token and aspects of its sequential placement. In: Atkinson, J.M., Heritage, J. (Eds.), *Structures of Social Action*. Cambridge University Press, pp. 299–345.
- Hoey, E.M., Kendrick, K.H., 2017. Conversation analysis. In: de Groot, A., Hagoort, P. (Eds.), *Research Methods in Psycholinguistics and the Neurobiology of Language: a Practical Guide*. Wiley, pp. 151–173.
- Hutchby, I., Wooffitt, R., 2008. *Conversation Analysis*. Polity.
- Jefferson, G., 1985. An exercise in the transcription and analysis of laughter. In: van Dijk, T.A. (Ed.), *Handbook of Discourse Analysis* (Vol. 3, Discourse and Dialogue). Academic Press, pp. 25–34.
- Jones, D., Drewery, R., Windle, K., Humphrey, S., de Paiva, A.F., 2024. Dementia prevention and the GP's role: a qualitative interview study. *Br. J. Gen. Pract.* 74 (741), e242–e249.
- Kang, B., Pan, W., Karel, M.J., McConnell, E.S., 2021. Rejection of care and aggression among older veterans with dementia: the influence of background factors and interpersonal triggers. *J. Am. Med. Dir. Assoc.* 22 (7), 1435–1441.
- Kemper, S., 1994. Elderspeak: speech accommodations to older adults. *Aging Cognit.* 1 (1), 17–28.
- Kent, A., 2012. Compliance, resistance and incipient compliance when responding to directives. *Discourse Stud.* 14 (6), 711–730.
- Kim, S., Liu, W., Daack-Hirsch, S., Williams, K.N., 2024. Communication patterns and characteristics of family caregivers and persons living with dementia: secondary analysis of video observation. *West. J. Nurs. Res.* 46 (4), 264–277.
- Kindell, J., Keady, J., Sage, K., Wilkinson, R., 2017. Everyday conversation in dementia: A review of the literature to inform research and practice. *International journal of language & communication disorders* 52 (4), 392–406.
- Kitwood, T., 1997. *Dementia Reconsidered: the Person Comes First*. Open University Press.
- Krummheuer, A., 2021. Unmet wishes. *Journal of Interactional Research in Communication Disorders* 10 (1), 5–29.
- Levinson, W., Lesser, C.S., Epstein, R.M., 2010. Developing physician communication skills for patient-centered care. *Health Aff.* 29 (7), 1310–1318.
- Li, L.W., McLaughlin, S.J., 2012. Caregiver confidence: does it predict changes in disability among elderly home care recipients? *Gerontol.* 52 (1), 79–88.
- Liddicoat, A.J., 2021. *An Introduction to Conversation Analysis*. Bloomsbury Publishing.
- Lindström, A., Weatherall, A., 2015. Orientations to epistemics and deontics in treatment discussions. *Journal of pragmatics* 78, 39–53.
- Ma, Z., Huang, L., 2025. Older adults' resistance to engaging in reminiscence therapy: a multimodal perspective. *Discourse Commun.* 19 (6), 997–1022.
- Mazeland, H., 2005. Conversation analysis. In: Brown, K. (Ed.), *Encyclopedia of Language and Linguistics*, vol. 3. Elsevier, pp. 153–163.
- Mortensen, A.H., Stojiljkovic, M., Lillekroken, D., 2021. Strategies to manage cognitive dissonance when experiencing resistiveness to care in people living with dementia: a qualitative study. *J. Adv. Nurs.* 78 (2), 503–513. <https://doi.org/10.1111/jan.15030>.
- Mundadan, R.G., Savundranayagam, M.Y., Orange, J.B., Murray, L., 2023. Language-based strategies that support person-centered communication in formal home care interactions with persons living with dementia. *J. Appl. Gerontol.* 42 (4), 639–650.
- Nichols, E., Szeoke, C.E., Vollset, S.E., Abbasi, N., Abd-Allah, F., Abdela, J., et al., 2019. Global, regional, and national burden of Alzheimer's disease and other dementias, 1990–2016: a systematic analysis for the global Burden of Disease Study 2016. *Lancet Neurol.* 18 (1), 88–106.
- O'Brien, R., Beeke, S., Pilnick, A., Goldberg, S.E., Harwood, R.H., 2020. When people living with dementia say 'no': negotiating refusal in the acute hospital setting. *Soc. Sci. Med.* 263, 113188.
- Pergolizzi, J.V., Raffa, R.B., Paladini, A., Varrasi, G., LeQuang, J.A., 2019. Treating pain in patients with dementia and the possible concomitant relief of symptoms of agitation. *Pain Manag.* 9 (6), 569–582.
- Psathas, G., 1994. *Conversation Analysis: the Study of talk-in-interaction*. Sage Publications.
- Rader, J., Barrick, A.L., Hoeffler, B., Sloane, P.D., McKenzie, D., Talerico, K.A., Glover, J.U., 2006. The bathing of older adults with Dementia: easing the unnecessarily unpleasant aspects of assisted bathing. *AJN Am. J. Nurs.* 106 (4), 40–48.
- Rampton, B., 2009. Linguistic ethnography, interactional sociolinguistics and the study of identities. In: Coffin, C., Lillis, T., Halloran, K.O. (Eds.), *Applied Linguistics Methods: a Reader*. Routledge, pp. 232–248.
- Regier, N.G., Choi, S., Gitlin, L.N., 2019. Rejection of care in persons with dementia. *Innov. Aging* 3 (Suppl. 1), S183.
- Roberts, T., 2009. Understanding ethnography. *Br. J. Midwifery* 17 (5), 291–294.
- Sabat, S.R., 1994. Language function in Alzheimer's disease: a critical review of selected literature. *Lang. Commun.* 14, 331–342.
- Savundranayagam, M.Y., Moore-Nielsen, K., 2015. Language based communication strategies that support person-centered communication with persons with dementia. *Int. Psychogeriatr.* 27 (10), 1707–1718.
- Savundranayagam, M., Ryan, E., Anas, A., Orange, J., 2007. Communication and dementia: staff perceptions of conversational strategies. *Clin. Gerontol.* 31, 47–63.
- Schegloff, E.A., 2007. *Sequence organization in interaction: A primer in conversation analysis*, (Vol. 1.). Cambridge University Press.

- Seedhouse, P., 2005. Conversation analysis and language learning. *Lang. Teach.* 38 (4), 165–187.
- Statista, 2024. Share of Population Aged 65 and Older in Taiwan, 1960–2060. Retrieved March 3, 2026, from. <https://www.statista.com/statistics/1112400/taiwan-share-of-persons-in-population-aged-65-and-older/>.
- Stivers, T., 2008. Stance, alignment, and affiliation during storytelling: when nodding is a token of affiliation. *Res. Lang. Soc. Interact.* 41 (1), 31–57.
- Stivers, T., Mondada, L., Steensig, J., 2011. Knowledge, morality and affiliation in social interaction. *Moral. Knowled. Convers.* 2011, 3–24.
- TADA (Taiwan Alzheimer's Disease Association), 2025. Understanding Dementia. Retrieved March 3, 2026, from. <https://www.tada2002.org.tw>.
- Tay, F.H.E., Thompson, C.L., Nieh, C.M., Nieh, C.C., Koh, H.M., Tan, J.J.C., Yap, P.L.K., 2018. Person-centered care for older people with dementia in the acute hospital. *Alzheimer's Dement.: Transl. Res. Clin. Interv.* 4, 19–27.
- Terada, S., Oshima, E., Yokota, O., Ikeda, C., Nagao, S., Takeda, N., et al., 2013. Person-centered care and quality of life of patients with dementia in long-term care facilities. *Psychiatry Res.* 205 (1–2), 103–108.
- Thomé, B., Dykes, A.K., Hallberg, I.R., 2003. Home care with regard to definition, care recipients, content and outcome: systematic literature review. *J. Clin. Nurs.* 12 (6), 860–872.
- Williams, K.N., Herman, R., Gajewski, B., Wilson, K., 2009. Elderspeak communication: impact on dementia care. *Am. J. Alzheimer's Dis. Other Dementias* 24 (1), 11–20.
- Williams, K., Kemper, S., Hummert, M.L., 2004. Enhancing communication with older adults: overcoming elderspeak. *J. Gerontol. Nurs.* 30 (10), 17–25.
- Williams, K., Shaw, C., Lee, A., Kim, S., Dinneen, E., Turk, M., et al., 2017. Voicing ageism in nursing home dementia care. *Journal of Gerontological Nursing* 43 (9), 16–20.
- Wooffitt, R., 2005. *Conversation Analysis and Discourse Analysis: a Comparative and Critical Introduction*. Sage.
- World Health Organization, 2025. *Dementia* (Fact Sheet) from. <https://www.who.int/news-room/fact-sheets/detail/dementia>. (Accessed 3 March 2026).
- Yang, C.C., Hsueh, J.Y., Wei, C.Y., 2020. Current status of long-term care in Taiwan: transition of long-term care plan from 1.0 to 2.0. *Int. J. Health Pol. Manag.* 9 (8), 363–364.